

Individual Case Safety Report (ICSR)

Pharmacovigilance Case Assessment — Drug Safety Associate Simulation

Task ID: task5 | Report Type: Spontaneous | Case Narrative Below

1. Case Identification

Report Type	Spontaneous report / training case
Case ID	_____ (assign per internal numbering)
Date of Report	_____
Reporter	_____ (Pharmacovigilance Intern / Drug Safety Associate)

2. Patient Information

Patient Initials	_____
Age	50 years
Sex	Male
Weight	_____
Relevant History	None reported

3. Suspected Drug Information

Drug Name	Ciprofloxacin
Dose	500 mg
Route	Oral
Frequency	Twice daily (BID)
Indication	Bacterial infection
Start Date	_____
Stop Date	Same day as onset (drug withdrawn)
Concomitant Medications	None reported

4. Adverse Event Narrative

A 50-year-old male patient was prescribed ciprofloxacin 500 mg orally twice daily for treatment of a bacterial infection. On the same day the patient began treatment, he developed generalized urticaria (hives), presenting as widespread, itchy, raised wheals across the skin, consistent with an acute hypersensitivity reaction. The temporal relationship between drug intake and reaction onset was immediate. Upon recognition of the reaction, ciprofloxacin was withdrawn and appropriate symptomatic treatment was administered. The patient's symptoms resolved and he made a full recovery, with no sequelae reported.

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5. Seriousness & Outcome

Onset	Same day after first dose
Seriousness	Non-serious — meets none of the 6 regulatory seriousness criteria (not fatal, not life-threatening, no hospitalization, no persistent disability, no congenital anomaly, not otherwise medically significant)
Outcome	Recovered / resolved

6. Expectedness Assessment

Expected. Urticaria and other hypersensitivity skin reactions are well-documented, labeled adverse effects of fluoroquinolone antibiotics, including ciprofloxacin. This reaction is consistent with the known safety profile of the drug and does not represent a novel or previously unreported event.

7. Causality Assessment — Naranjo Probability Scale

#	Question	Answer	Score
1	Previous conclusive reports on this reaction?	Yes	+1
2	Did ADR appear after suspected drug given?	Yes	+2
3	Did reaction improve on drug discontinuation?	Yes	+1
4	Did reaction reappear on re-administration?	Not done	0
5	Alternative causes could explain reaction?	No	+2
6	Did reaction reappear with placebo?	Not done	0
7	Drug detected in blood/fluid at toxic levels?	Not done	0
8	Dose-related worsening/improvement observed?	N/A	0
9	Similar reaction to same/similar drug before?	Unknown	0
10	Confirmed by objective evidence?	Yes (clinical observation)	+1

Total Naranjo Score: 7 → Category: Probable

8. WHO-UMC Causality Category

Probable. There is a reasonable time relationship between ciprofloxacin intake and the onset of urticaria. The reaction is unlikely to be explained by the patient's underlying condition or other medications, as none were reported. Clinical improvement followed withdrawal of the drug. Rechallenge was not performed, so the reaction cannot be classified as "Certain."

9. Severity & Case Classification Summary

Seriousness	Non-serious
Expectedness	Expected

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Clinical Impact	Low — self-limiting reaction, resolved fully following prompt drug withdrawal and symptomatic treatment; no lasting harm to patient
Regulatory Reporting Priority	Routine (non-expedited) — case does not meet criteria for expedited/serious ADR reporting

10. Overall Conclusion

Based on the temporal relationship between drug administration and reaction onset, resolution of symptoms following drug withdrawal, the Naranjo score of 7 (Probable), and WHO-UMC classification of Probable, **ciprofloxacin is assessed as the probable causative agent** for the generalized urticaria observed in this patient. The event is classified as **non-serious and expected**, warranting routine documentation in the pharmacovigilance database without need for expedited regulatory reporting.